

Buying Health Differently

What it would mean for a county cooperative to hold the health care dollar — and to buy, instead of services delivered, *a named person's plan being met*. Written for all four of the county's payment groups, not just employers.

For the WWAH organizing team **Status** a proposal for discussion, not a decision

Revised 26 August 2026

START HERE

The short version

THE WHOLE IDEA IN ONE PARAGRAPH

In American health care, whoever holds the money decides what gets paid for. Today that is a carrier, a federal agency, or a state agency — and what they pay for is *activity*: visits, procedures, codes. If a county cooperative held that money instead, or acted as agent for those who do, it could decide to buy something different: one named person's care plan being met, confirmed by that person. Everything else in this document follows from that single change.

Seven things worth knowing before you read further.

1. **We would not need anyone's permission.** This does not require a hospital to reform itself. It requires a buyer to buy differently.

2. **There are four payment groups, and only one variable separates them.** Employers, Medicare, Medicaid, and the uninsured differ in who holds the money and what they are permitted to buy with it. The order, the teams, the verification and the safeguards are identical in all four. So this is one design with four *adapters*, not four designs.
3. **There is a model, and it is not from health care.** Haier, the Chinese appliance company, removed its middle management and rebuilt itself as thousands of small units contracting directly with named customers. The approach is called RenDanHeYi, and it has never been applied systematically to medical care.
4. **Health care has already done it three times without naming it.** The Alaska Native health system, the Esther network in Jönköping, Sweden, and Buurtzorg in the Netherlands all work this way. None of them had to become small to do it.
5. **The money design is the whole thing.** Roughly 70% pays teams simply to exist and be available — steady and unconditional, so nobody gambles with their income. About 20% attaches to a specific person's plan. About 10% is a shared bonus against a target the team can actually move. Those numbers are a dial, not a recommendation.
6. **It must be built as a series of experiments, and the first one moves no money at all.** Everything visible, nothing at risk, for the first several months.
7. **Half of what makes people healthy is not medical, and needs a different mechanism.** That half belongs to neighborhoods, and is discussed from Part 8 onward.

HOW TO READ THIS

Parts 1–7 are about care and the cooperative. Parts 8–10 are about public health and neighborhoods, which need a parallel but different structure. Part 11 is an honest list of what could sink it, and Appendix A gives the employer mechanism in detail. Every unfamiliar term is defined in the glossary and again on first use. The diagrams can be skipped without losing the argument — the text carries it.

PART 1

What "the medical system" means here

IN ONE SENTENCE

Not doctors' offices — clinics, hospitals, pharmacies, laboratories, imaging, and the therapies delivered by professionals who are neither physicians nor nurses.

This matters enough to state before anything else, because the alternative reading collapses the whole proposal back into "clinic visits," which is the thing we are trying to get away from. When this document says the medical system, it means all of it:

- **Clinics** — primary care and specialty.
- **Hospitals** — inpatient, emergency, procedural.
- **Pharmacies** — who see many members far more often than any physician does.
- **Diagnostics** — laboratory and imaging.
- **Therapies delivered by non-physician, non-nurse professionals** — physical, occupational and speech therapy, behavioral health, nutrition, dental care.
- **Community health workers** — who work in the neighborhood rather than the building.

That last group is not a footnote. Much of what today's payment system covers *worst* lives there, which makes it a substantial part of the argument for changing what gets bought rather than an afterthought to it.

The word we use: care team

Throughout this document, a care team is **any small group a member actually meets**. A clinic pod, a pharmacy, an imaging suite, a physical therapy practice, a community health worker team. A support team is one a member never meets — billing, data, supply, training.

We avoid the word "clinic" as a shorthand for all of them, because it silently excludes the pharmacy and the imaging suite and makes readers picture a building with a doctor in it. We also mostly avoid the technical term microenterprise, using it only where the economic property is the point: that a care team has its own account and can accept or decline a piece of work.

A DECISION WORTH NOTICING

A hospital does not participate as a single entity. It participates as **one or more service lines, each its own care team**. This is the Haier move — break the giant into units and contract with the units — and it is a materially different proposition to put to a health system than asking the institution to sign one agreement. It is also the only version that does not simply reproduce instructions flowing downward, which is the problem we are trying to solve.

PART 2

Who holds the money decides what gets bought

IN ONE SENTENCE

The person receiving care is the object of every contract in the system and a party to none of them, and that is a consequence of where the money sits.

Follow a commercially insured dollar. An employer and its employees pay a premium to a carrier. The carrier holds that money and therefore holds the contract — it decides what is covered, what is paid for, and at what rate. It negotiates with a health system, which is large partly because being large improves the rate it can negotiate. The health system employs the clinicians and passes down budgets and targets. A clinic delivers care to a person. The person receives it.

Notice what the person does *not* do. They sign nothing that anyone is paid against. Their satisfaction may be surveyed, but the survey pays for nothing. Money flows on activity, upward, and the one human being the whole apparatus exists for is the only participant with no contract.

The Medicare and Medicaid chains differ in their details — a federal or state agency stands where the carrier stands — but the structural fact is the same in all three: the person is the object, not a counterparty.

Why consolidation keeps happening

It is tempting to treat the dominance of large, top-down health systems as a failure of imagination or values. It is more useful, and more accurate, to notice that consolidation is *rewarded*. Buy a physician practice, attach a facility fee, and the identical visit bills more. Grow large enough and the commercial rate for the same care rises through negotiating leverage. Carry the apparatus of claims, coding, compliance and the electronic record, and scale is the only way to spread its fixed cost.

None of that is a philosophy anyone chose. It is a behavior the payment system reinforces on a short cycle with a real payoff — which is exactly why arguing against it does not work. You cannot argue someone out of a reward schedule. You can only change what is rewarded, and only the buyer can do that.

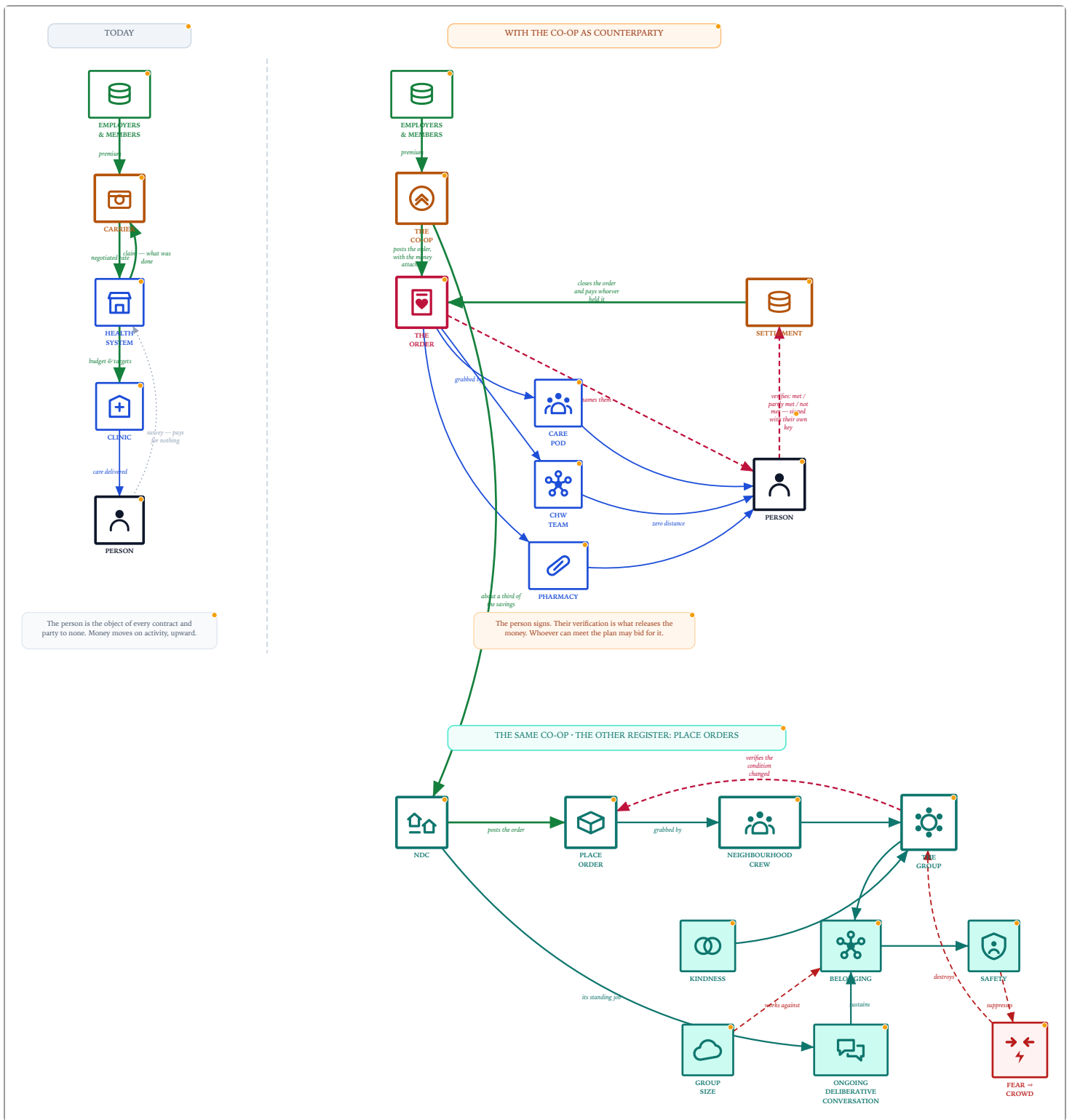


Figure 1 — Where the money sits. The same dollar in two positions. On the left, today: the carrier holds it, and the person at the bottom is connected to the system by a single line marked "care delivered." On the right, the cooperative holds it, posts one named person's plan as an order, and that person's own signed confirmation releases the payment. Lower down on the right is the second register discussed in Part 8.

- Green box: where the money comes from
- Orange box: who holds it
- Blue box: who does the work
- Black box: the person
- Red box: the order, and verification

A model from outside health care

IN ONE SENTENCE

A Chinese appliance manufacturer removed its middle management and rebuilt itself as thousands of tiny businesses, each contracting directly with named customers — and health care has quietly done the same thing three times without the vocabulary.

Haier employed something like eighty thousand people when its chairman, Zhang Ruimin, began dismantling its management hierarchy. The approach is called **RenDanHeYi** (人单合一). Each of the four characters is a design constraint:

CHARACTER	MEANING	WHAT IT DEMANDS
Rén 人	the person as a moral agent	not a role, not a headcount, not a "covered life"
Dān 单	a concrete single order	one identified person's specific need — never a market segment or a volume target
Hé 合	active merging	something people do continuously, not a structure someone installs
Yī 一	oneness achieved	the merge actually succeeded, and can be evidenced

The hard word is **Dān**. In appliances it is obvious: a named person ordered a specific machine. In medical care, what is the single order?

The most useful answer is that **a person's shared care plan is the order**. Not the visit, not the episode, not the attributed life. A named person's plan is concrete, singular, verifiable, and the person can confirm whether it was met. Everything else in health care that looks like an order — a referral, an authorization, a claim — is really an order from the institution to itself, with the person as its object.

Three proofs that already exist in care

- **Southcentral Foundation's Nuka system** in Alaska calls the patient a *customer-owner*. That is Rén, precisely: a person with standing rather than a body with an account number.
- **The Esther network** in Jönköping, Sweden, invented a fictional elderly woman named Esther and asks of every proposed improvement: what is best for Esther? That is Dān, invented independently and given a first name.

- **Buurtzorg** in the Netherlands runs home nursing with roughly fifteen thousand nurses in self-managing teams of about a dozen, supported by a back office of perhaps fifty people, with essentially no managers in between.

None of the three had to become small. Southcentral runs a whole regional system; Jönköping is a county. The lesson is not that big is bad — Haier is still enormous. The lesson is that *big plus instructions-flowing-downward* is the problem, and the two can be separated.

Where W. Edwards Deming fits

Deming's tradition — and the work of Associates in Process Improvement in *The Improvement Guide* and *Quality as an Organizational Strategy* — gives us the discipline of viewing an organization as a system with a single aim, mapping how its processes link together, and knowing who owns each process and how it is measured. It also supplies two hard rules used later: never implement a change you have not tested at increasing scale, and never treat ordinary random variation as a signal.

What that tradition leaves unresolved is *who holds the aim*. It assumes a management that can be trusted to hold it on everyone's behalf. RenDanHeYi is the only serious answer we have found that removes that assumption without dissolving the system: the aim comes from the named person, not from a manager.

PART 4

Four payment groups, one order

IN ONE SENTENCE

The four groups differ in exactly one variable — who holds the money — so the cooperative needs four *adapters*, not four designs.

A cooperative serving a whole county cannot stop at employers. Employer-sponsored coverage is a minority of any American county's population — likely around half of Whatcom at most — with Medicare, Medicaid and the uninsured making up the rest. **These proportions are estimates and should be replaced with real local figures before this brief is used to persuade anyone;** the mix determines which adapter matters most, so it is worth getting exactly right early.

The encouraging structural fact is that the order, the care teams, the verification, the microcommunity, the catalytic test and the safety ramp are *identical* in all four cases. Only the way money arrives changes.

GROUP	WHO HOLDS THE MONEY	THE REALISTIC ADAPTER	THE CATCH
Employers	self-funded employers	Cooperative as contracting agent for a pool. See Appendix A.	Employers can leave the pool after one bad year — and the pool is the leverage.
Medicare	CMS (federal)	An ACO — already an attributed panel, a spending benchmark, shared savings, and no insurance license.	Benchmark ratcheting : beat the target and next year's target drops.
Medicaid	WA Health Care Authority, via managed care organizations; the regional ACH	A delegated arrangement with a managed care organization, or work through the Accountable Community of Health.	Churn — people cycle off eligibility, and a plan needs continuity. Rates are low.
The uninsured	nobody	Avoided cost — hospital community benefit, county funds, a low-cost membership.	No premium means no counterparty lever at all.

Three consequences worth arguing about

Medicare arrives with the machinery we designed. An accountable care organization is already an attributed panel plus a spending benchmark plus shared savings plus risk adjustment, and it requires no insurance license. It is the closest existing federal vehicle to this design. But its benchmark ratchets: succeed and you are rewarded with a harder target next year. That is a reinforcing loop pointed *against* you and a direct failure of the catalytic test in Part 6. Enter with eyes open and model several years rather than one.

Medicaid is the best first payer for the neighborhood side, not the worst. It is the only payer that already buys community health workers, care coordination and some housing support. Employers will not buy a crosswalk. If the place register in Part 8 needs a first funder, it is Medicaid.

The uninsured are already covered by half the design. A place order asks nothing about insurance status, and the neighborhood charter says *all* the people in the place. So they are served by the place register from day one. What they lack is a payer for the person register — and there the counterparty is the hospital and the county rather than an employer, because a hospital would rather fund a community health worker team than absorb the emergency visits, and community benefit is money it must spend regardless.

Someone turns 65. Someone loses a job. Someone's income rises. Today the care arrangement shatters at each of those boundaries, because it belongs to the payer. If the plan belongs to the *person*, it survives: the plan persists and only the adapter changes. No current payer can offer that, and it is not a feature anyone had to add — it is a direct consequence of indexing the order to a person instead of to a contract.

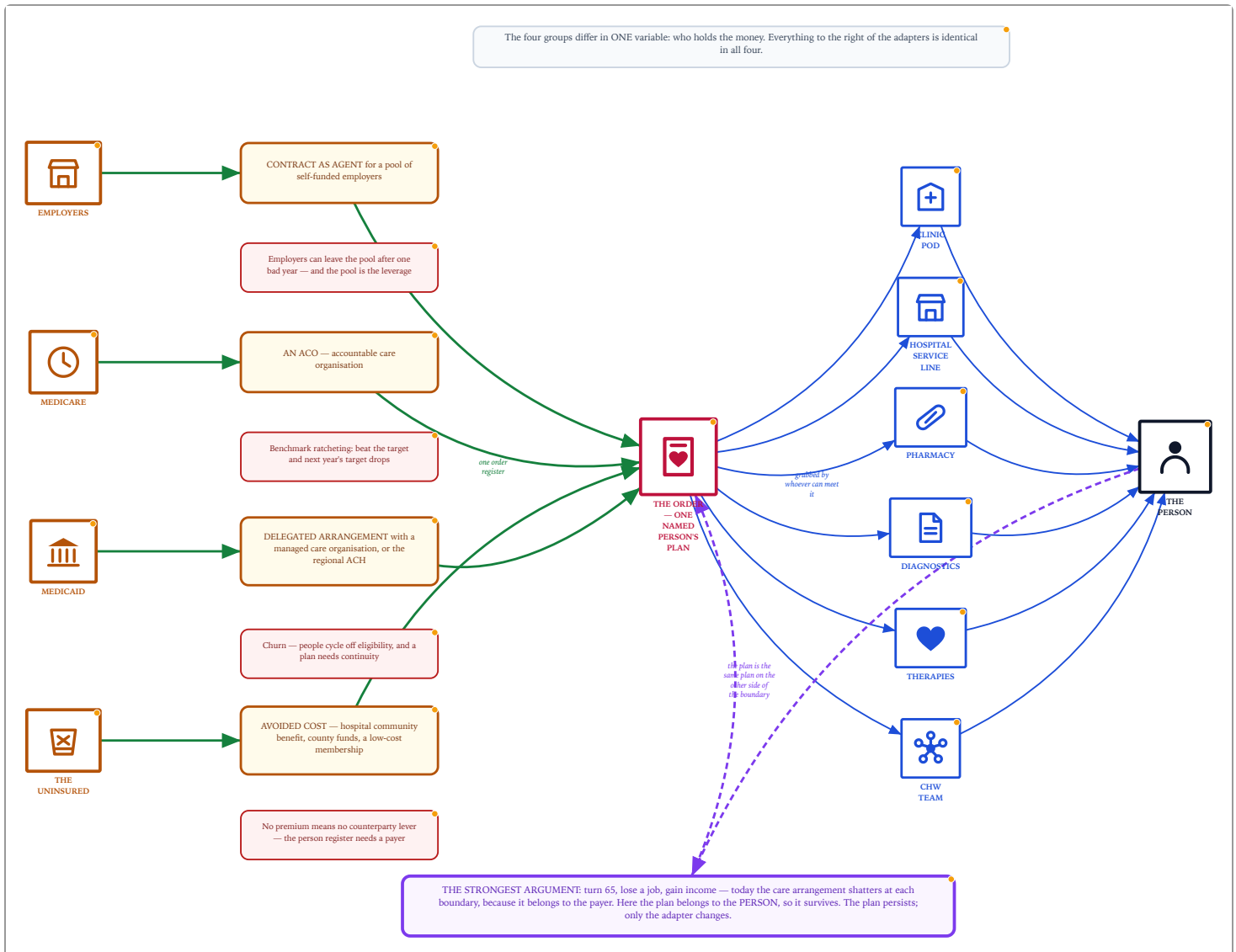


Figure 2 — Four payment groups, one order. Money enters on the left through four different adapters, each with its own catch in red. Everything to the right of the adapters is identical in all four cases: one order, the care teams that can take it, the person. The purple band at the bottom is the continuity argument.

 payment group and adapter
 the order
 a care team
 the person
 the catch

Three positions, in every payer's world

IN ONE SENTENCE

For each payment group the cooperative can convene, contract as agent, or bear risk — and that choice, not the payer, decides whether the workbench is a coordination system or a contracting system.

The same three positions recur in every payer's world:

	CONVENE	CONTRACT AS AGENT	BEAR RISK
Employers	a shared table and shared measures	agent for a pool of self-funded employers	become a licensed carrier
Medicare	a provider coalition	an ACO with shared savings	sponsor a Medicare Advantage plan
Medicaid	participate in the regional ACH	delegated subcontract with a managed care organization	become a managed care organization
Uninsured	coordinate charity care	steward pooled community-benefit and county funds	—

Convene

Holds relationships and a shared table. It can agree measures, publish comparisons, build trust, run improvement collaboratives. It cannot change what gets paid for, cannot require anyone to verify anything, cannot open a piece of work to a better offer, and cannot end an arrangement that is not working. Its software is a **coordination system**: shared plan, shared identity, shared measures, no settlement. Its characteristic failure is the one everybody has seen — high-quality agreement, a published report, and a baseline that does not move.

Contract as agent

Someone else keeps the risk they already carry; the cooperative controls what is bought and who is paid for it. This is the counterparty position *without* a license, and it is the reachable one in three of the four groups. Its software is a **contracting system**: order register, contracts, verification, settlement ledger.

Bear risk

The cooperative holds the money outright. Everything the agent position unlocks, plus the price and the surplus — at the cost of a license, reserves, actuarial capability and a state regulator. Its characteristic failure is becoming the carrier it set out to replace, and then rediscovering why carriers behave as they do.

A JUDGEMENT, OFFERED FOR ARGUMENT

Contract as agent, in every group where it exists. It buys the counterparty seat without buying a license. And the employer version is testable quickly: the broker in this group can likely establish in an afternoon whether enough self-funded employers exist locally to form a pool. That single question gates the fastest adapter.

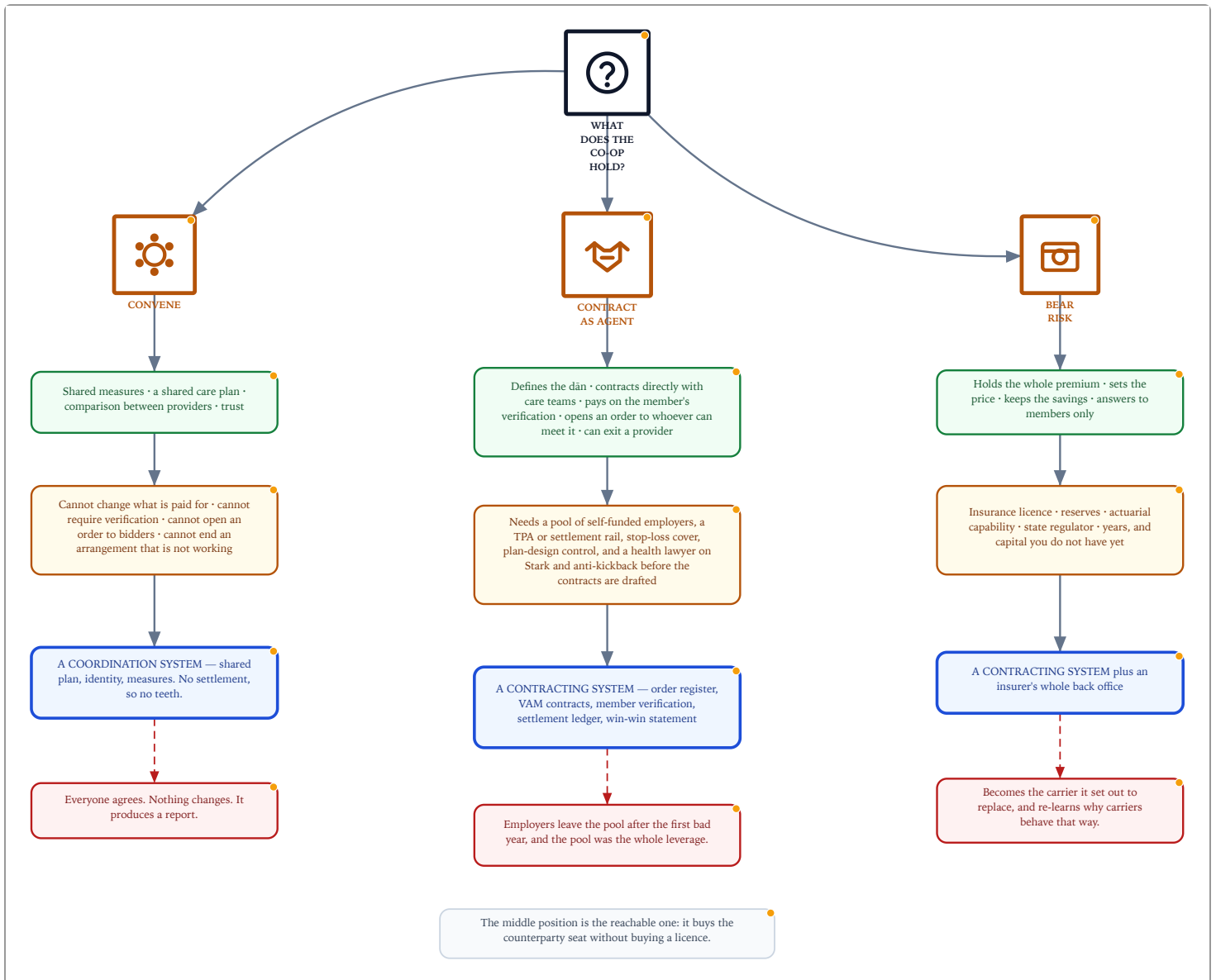


Figure 3 — What does the cooperative hold? Drawn for the employer case as the worked example; the same three positions recur in each of the other groups. For each: what it unlocks (green), what it costs or forecloses (amber), what kind of system the workbench then is (blue), and how it characteristically fails (red).

How the money would move, and how it would match performance

IN ONE SENTENCE

Three streams out of one pooled fund — a large unconditional one that pays teams to exist, a middle one attached to a named person's plan, and a small shared bonus — with guards on each to stop the mechanism corrupting itself.

Whatever the adapter delivers — employer contributions, a Medicare shared-savings settlement, a Medicaid capitation, pooled community-benefit dollars — arrives in one pooled fund held by the cooperative, in a trust or third-party administrator account. The cooperative takes a transparent administration fee rather than a margin on the flow; that difference is most of what separates a cooperative from a carrier. The fund then splits three ways.

Capacity — roughly 70%, no conditions

Pays a care team simply to exist and be available for the people on its panel. Steady, monthly, predictable. This is the stream that lets a community health worker team or a clinic pod take responsibility for a person's plan without anyone becoming an entrepreneur. Most physicians sold their independence voluntarily and many experienced it as a relief; a design that requires clinicians to gamble with their income is asking for the thing they already declined. Buurtzorg's nurses are salaried — the self-management is in the work, not the paycheck.

The order — roughly 20%, a floor plus verification

Attached to one named person's plan. A floor is paid on delivery. The person's own confirmation — met, partly met, or not met, signed with their own key on their own record — releases the remainder.

A SAFETY RULE THAT SHAPES THE MECHANISM

Verification must *raise* payment above a floor and must never gate it to zero. If a missing signature withheld money, every team would have a financial reason to press a sick person for one, and the sickest people are the easiest to press. **Silence is neutral:** a member who says nothing costs nobody anything. That single property is the difference between a verification system and a collections system.

Shared upside — roughly 10%, against a leading target

A bonus agreed in advance against a target the team can actually move this week — days from asking for help to having a plan, the share of members with a current plan, follow-ups completed within two weeks. Not readmission rates, which are lagging, confounded by everything, and

involve too few people at neighborhood scale to mean anything.

It is paid to the whole microcommunity bound to that person — every team that had to succeed for the plan to be met — and never to a ranked individual. That matters because Deming spent his last decade attacking incentive pay, ranking, and management by objective, and a bonus on a target is recognizably all three. The defense is that the target comes from a named person rather than a boss, and that the unit of reward is the group around that person. That is a real defense. It is not obviously a complete one, and the team should decide this consciously rather than inherit it.

Three guards

- **Risk-adjusted order value.** A harder plan carries more money, so taking the easy work stops being the winning move. Without this, opening work to bidders produces cherry-picking — the best-documented failure of every such scheme.
- **Chart it before you pay on it.** A measure does not qualify as a payment trigger until it has been on a control chart long enough to show whether its movement is a real signal or ordinary random variation. Paying against a stable process month to month is literally paying for randomness, and Deming showed what that produces: tampering, and a system made worse by people trying to improve it.
- **The win-win value-added statement.** Haier's replacement for the conventional profit-and-loss account. It counts ecosystem value — members served, resources drawn in, total cost trend against the county — and decides how big the shared pot is before it is divided. Nothing in health care has an equivalent; it is the instrument we are most obviously missing.

The catalytic test

THE TEST TO APPLY TO EVERY DOLLAR

Does receiving this reduce the need for it, or increase it?

This is the distinction between a *catalytic* foundation, which changes a system so the need declines, and a *provider of consumables*, which funds a need that returns next year, larger. Applied to money it is structural rather than moral, and every stream carries its answer:

STREAM	ANSWER
Capacity	Enabling — the floor the others stand on. Neither self-terminating nor consumed.
The order	Self-terminating when the plan is met.

STREAM

ANSWER

Shared upside	Self-terminating <i>only</i> if the target is charted and the share equalized.
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Savings share	Self-terminating <i>only</i> if expected cost is risk-adjusted.
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Plans that never close	Consumable by design — and that is correct.
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TWO CONSEQUENCES WORTH ARGUING ABOUT

Risk adjustment is structural, not a fairness nicety. Savings arise against *expected* cost. Adjust the expectation for risk and money flows toward the neighborhoods with the most headroom, which are the sickest. Fail to, and the healthiest neighborhoods look most efficient and collect the most — compounding the very inequality the design exists to reduce, and looking like merit the entire time.

Some plans never close, and that is not failure. Brain, body and emotional injury can leave someone permanently unable to support themselves. For them the loop never reaches zero. Those plans are carried entirely by the capacity stream: no verification gate, no order payment, and no implication of failure when the plan does not close. Without this category the mechanism quietly penalizes the most damaged people for not getting better — efficiently, and invisibly.

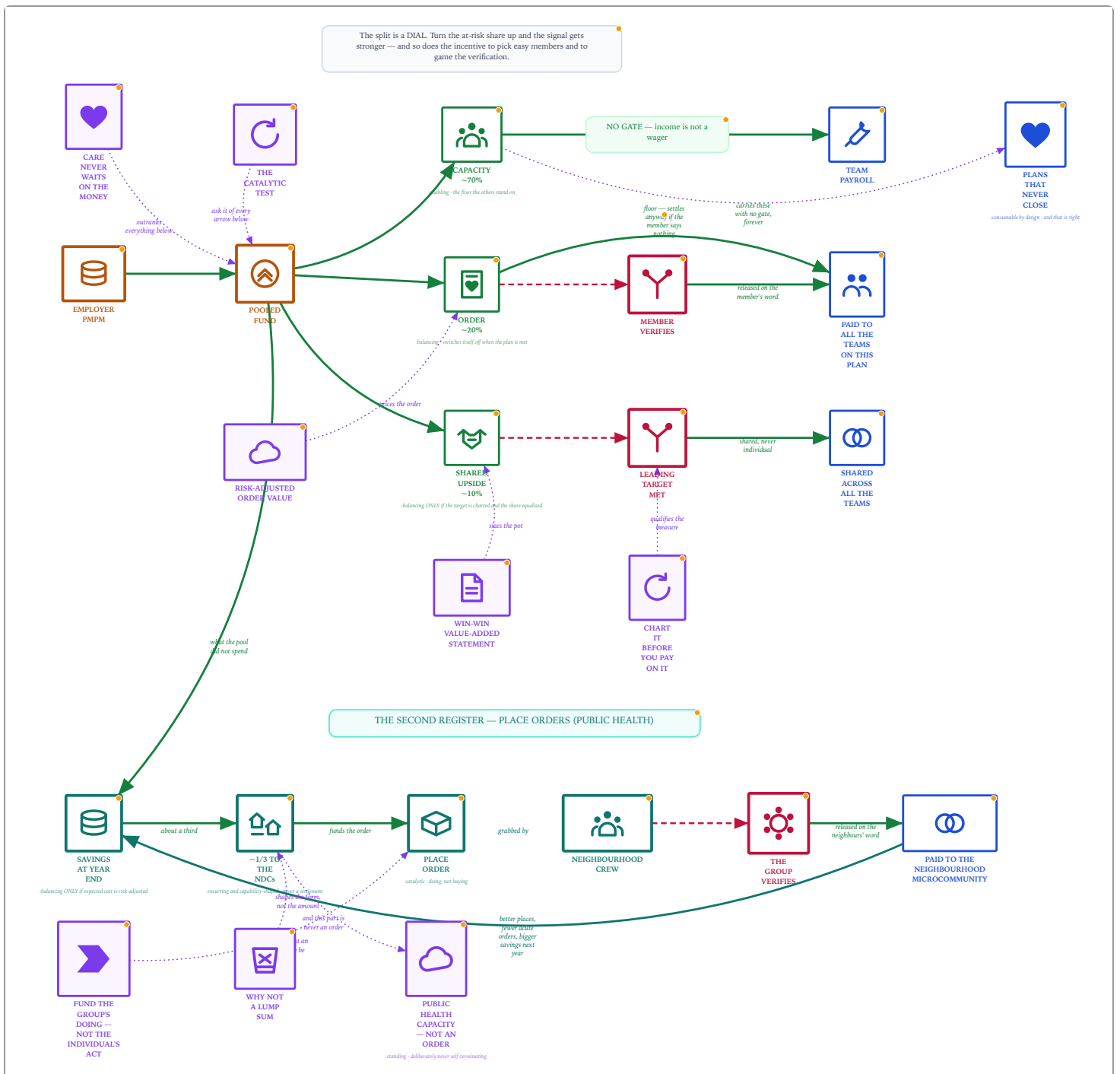


Figure 4 — How the money moves and what makes it move. The pooled fund at left splits into three streams. Two pass through gates (red) before reaching the people who did the work; one deliberately does not. Purple boxes are guards. The lower band is the neighborhood register described in Part 8.

■ where money is held
 ■ a stream of the dollar
 ■ a gate
 ■ who receives it
 ■ a guard

PART 7

Doing this without hurting anyone

IN ONE SENTENCE

This is an experiment on how real people's care is arranged and paid for, so it is built as a ramp of increasing scale with stopping rules written down in advance — and the first step moves no money at all.

Deming and API never *implement* a change; they test it at increasing scale under widening conditions, and the size of the first test is set by how much is unknown and how much a failure would cost. Here a failure could cost someone their health. So three rules hold across every step, without exception:

1. **Care never waits on the money.** No clinical decision is ever contingent on settlement state. The experiment runs beside care, never in front of it. If the ledger is broken, wrong or disputed, the care still happens and the argument happens afterward. This rule outranks every other rule in this document.
2. **Additive only.** Early experiments *add* a service — a community health worker contact, a plan conversation, a follow-up call. Nothing existing is removed, reduced or replaced until the additive version has run long enough to show what it does. You cannot easily harm someone with a service they did not have before. You can very easily harm them by taking one away.
3. **Silence is neutral.** A member who does not verify costs nobody anything.

The ramp

STEP	WHAT CHANGES	WHAT STOPS IT
E0 · Shadow	The whole ledger runs in parallel and moves no money. Orders posted, verifications collected, targets computed — everyone paid exactly as today. Three to six months.	The shadow ledger disagrees with the real one in ways nobody can explain.
E1 · One team	One care team, one condition, about thirty consenting members. Additive only. Capacity plus an order floor. Nothing at risk.	Any member harmed; any member confused about who is responsible for their care.
E2 · Verification live	About a hundred members. Verification now releases the remainder above the floor.	Any member reports being pressed to verify; or the verification rate climbs toward 100%.
E3 · Microcommunity	Two or three teams bound around one person's plan, with a small shared upside against a charted target.	A team starts optimizing the target instead of the person; declines cluster in one group.

STEP	WHAT CHANGES	WHAT STOPS IT
E4 · Open work	More than one team may bid for an order, at risk-adjusted value.	The sickest members wait longer for a team to take their order than anyone else.

WHY EO MATTERS MOST

The shadow step is where everything currently hidden becomes visible at zero risk to anybody: what a plan actually costs, who actually moves it, and how often a member says the plan was not met. That is most of the education this whole effort promises, available before anyone's care or income changes.

Two more things make the stopping rules real rather than decorative. There must be a named **safety review** — a person and a small group with authority to halt any step and no financial interest in its continuing, seeing the signals directly rather than filtered through the people running the experiment. And this should go to an **ethics review board** whether or not it legally counts as human-subjects research. It is protective for members, protective for the cooperative, and it is the difference between an experiment and something done to people.

One counter-intuitive tripwire is worth explaining, because it will look like success: **a verification rate approaching 100% is a warning, not an achievement**. Nobody meets every plan. A near-perfect rate means members have learned what answer is wanted, which is the moment verification stops carrying information and starts laundering it.

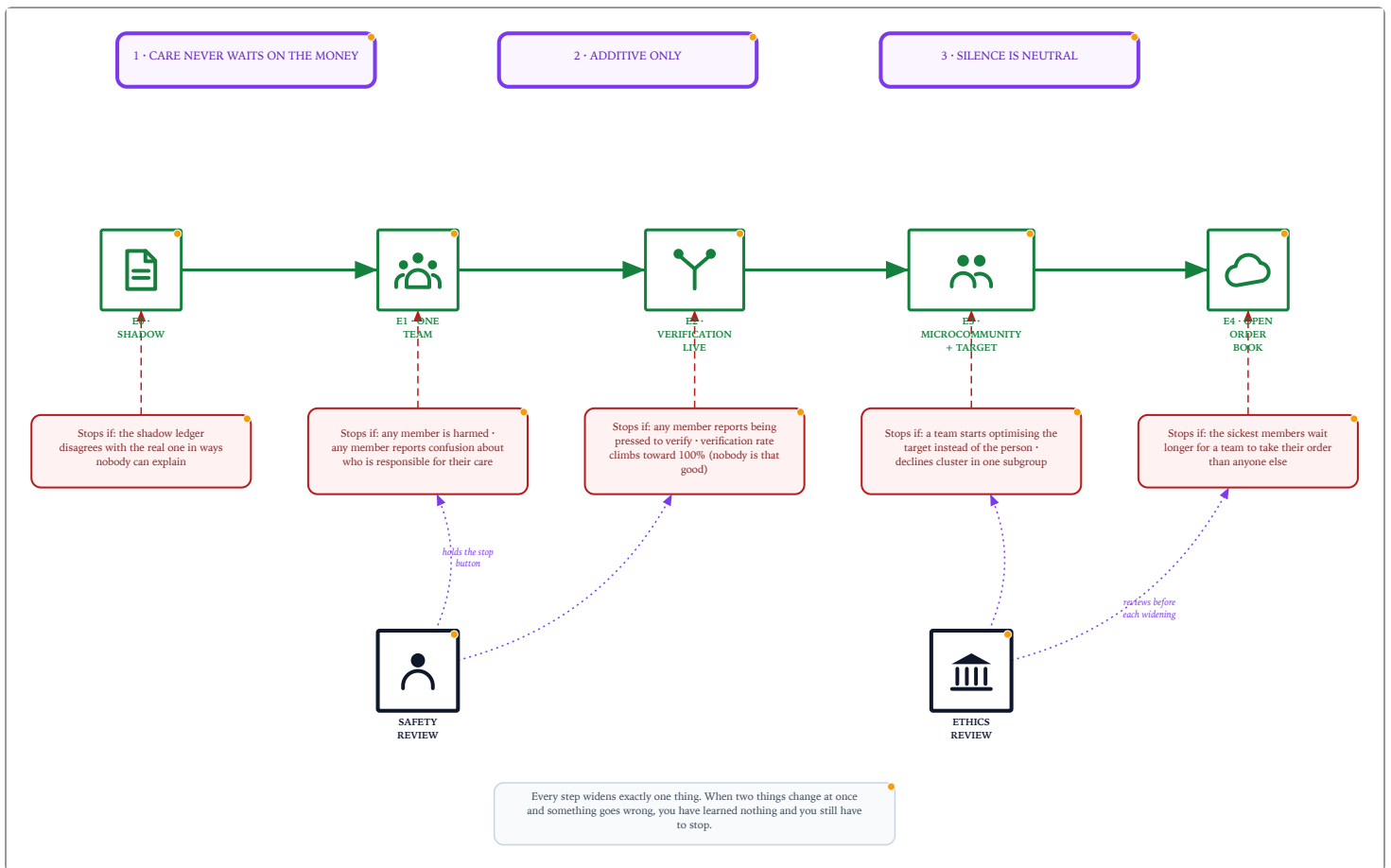


Figure 5 — A ramp of experiments, and what stops each one. Purple across the top: the three standing rules. Green: the five steps. Red: the stopping rule for each, written before it starts. Black: who is watching, with no stake in the answer.

PART 8

The other half: places, not just people

IN ONE SENTENCE

Everything above is indexed to a person, which is why it fits medical care and cannot carry public health — so public health gets a parallel structure indexed to a *place*, verified by a group of residents rather than an individual.

The mechanism described so far is person-indexed from end to end. Medical care fits this natively. Public health does not, and the reason is structural: its unit is a population or a place, its benefits cannot be withheld from anyone, and its successes are non-events. Nobody signs a form confirming they did not catch cholera. The epidemiologist Geoffrey Rose named the underlying

problem — a preventive measure that delivers large benefits to a population offers very little to any individual within it. That is why person-indexed contracting cannot carry public health. It is the wrong index, not a badly tuned one.

The separation is also protective. Public health inside a machine that pays per event is always first to be cut, because its success looks like nothing happening.

The place order

If a person cannot be the index, a place can. Not "reduce obesity in Whatcom County" — an abstraction nobody can own or verify. But: *this block has no safe crossing to the school*. Concrete, singular, visible, and the people who live there can say whether it changed.

That is the order for public health: **a named condition in a named place**. It gives the cooperative a second register on the same workbench — person orders verified by the person, place orders verified by the people who live there.

Who pays for it, and the wrong-pocket problem

The cooperative has a direct financial interest in the condition of places, because those conditions drive what care costs — while capturing none of the savings from anything the county paid for. This wrong-pocket problem is what kills prevention everywhere.

The proposal on the table is that **about a third of the cooperative's savings returns to neighborhood development corporations as a place-order fund**. Better places produce fewer acute orders, which produce larger savings, a third of which funds better places. The loop closes, and the thing is self-funding rather than philanthropic. Medicaid is the natural additional funder here, being the only payer that already buys the social layer.

DOING, NOT BUYING

A place order funds the neighborhood *doing* something, not the cooperative *buying* something. The distinction is not the amount — it is whether the unit of funding is a collective project or a transaction. Money buys materials, tools and skilled accompaniment; the labor is the group's own. A useful test: if a contractor could execute it faster and better, it was specified as a purchase and should be re-cut.

And it must fund **the group's doing, never the individual's act**. Paying one neighbor per check-in on another is a piece rate on something that only worked as a gift, and it reliably destroys the reciprocity it meant to encourage.

Why a group, and not just any group

Plural verification looks like it invites every collective-action problem: who speaks for the block, whose signature counts, capture by the loudest. The resolution is more specific than "build community."

Kindness makes a group. A group produces belonging. Belonging produces safety. Safety suppresses fear — and fear is what manufactures a *crowd*. A crowd is not a large group; it is the destruction of one. So the failure mode is caused by fear, not by numbers, which means procedural fixes like voting rules do not reach it. The work is upstream and relational.

- **Size is a design constraint, not a preference.** Group size works directly against belonging. Since a place order is verified by the residents of an area, belonging sets the maximum size of that area.
- **Unanimity is a warning sign.** Mary Parker Follett, writing in 1918, described the living group as one that *interweaves difference* into something no member brought in — as against a crowd, which achieves false unity by suppressing difference. A verification that comes back unanimous is evidence difference has been suppressed. Nineteen households of which eleven say improved, five say no change and three say worse is the verification. Collapsing that to a yes or no throws away the only signal that would tell you the group has stopped being able to disagree.

THE BOUNDARY CONDITION THAT BELONGS IN A CHARTER

A neighborhood group is self-selecting in **who does the organizing** and must never be self-selecting in **who is cared for**. The moment those collapse together you have a private club wearing the clothes of a commons — which is the precise difference between a neighborhood development corporation and a homeowners' association.

PART 9

Sorting what belongs where

IN ONE SENTENCE

Two questions — can anyone be kept out, and does my use diminish yours — sort every social determinant of health into public goods, shared commons, or private goods, and each needs a different buyer, verifier and kind of money.

The classification is not a matter of politics. Given two facts about a good, the cell follows, and so do the consequences:

	NOBODY CAN BE KEPT OUT	ACCESS CAN BE LIMITED
My use doesn't diminish yours	Public good. Governed, tax-funded, never an order. Air and water quality, disease surveillance, emergency response, codes and enforcement.	Club good. The cooperative's own instruments — membership, the workbench itself.
My use diminishes yours	Commons. Neighborhood level. Place orders plus unconditional capacity. The food environment, community health worker time, eyes on the street, tenure stability, road capacity at peak, groundwater.	Private good. Person orders. The dwelling unit, groceries, medications, a specific clinical service, cash.

Split the determinant first

Almost nothing is a single good. **Housing is not one thing:** the dwelling unit is private, habitability codes are public, and the block's condition and tenure stability are a commons. Assign "housing" as a whole and you get it wrong three ways at once. Water quality is a public good while an aquifer is a commons. A street is a public good at ten in the morning and a commons at five in the afternoon, because congestion is what makes a thing rival.

What makes something necessarily a commons

Any one of these is sufficient:

- **It is anti-rival** — it grows with use, and enclosing it destroys it. Knowledge, skill, trust, norms. Teaching someone a skill increases the stock rather than depleting it; charge admission and the mechanism that created the value stops working.
- **Its production has a size ceiling** — it cannot be made at scale at any budget. Belonging is the proof case: since group size works against belonging, belonging is the one determinant of health that is *provably* undeliverable at state scale. A government can fund the conditions for it and can never produce it. This is the strongest argument available that neighborhood organizations are structurally necessary rather than merely admirable.
- **It is co-produced** — the beneficiaries are the producers. Mutual aid, informal care, watchfulness on a street. These cannot be delivered to passive recipients, which is why a service window cannot make them and a group can.

The mirror: something must be a **public good** if free-riding would defeat local voluntary provision, or if it is adversarial (enforcement can never be co-produced by the party being inspected), or if its scale exceeds any possible group, or if its failure would be catastrophic and irreversible.

Two ways to get it wrong

Enclosure — selling what only worked when shared — is the familiar one. The second is the live risk in this design: **crowding out**. Pay people for what they were already doing out of neighborliness and the reciprocity norm is replaced by a transaction that is worse and costs more than what it displaced.

WHERE THE ARGUMENT WILL ACTUALLY HAPPEN

The sorting frame is stable; which cell a particular thing goes in is contestable — because whether something can be excluded is usually an institutional choice rather than a fact of nature. A road is non-excludable until someone installs tolling. Enclosure is literally the act of making a commons excludable. So when someone says "housing is a private good," that is not a description of reality; it is a claim about how things should be arranged, wearing the clothes of a technical classification. The frame's value is that it forces such claims into the open. It does not settle them, and it is not supposed to.

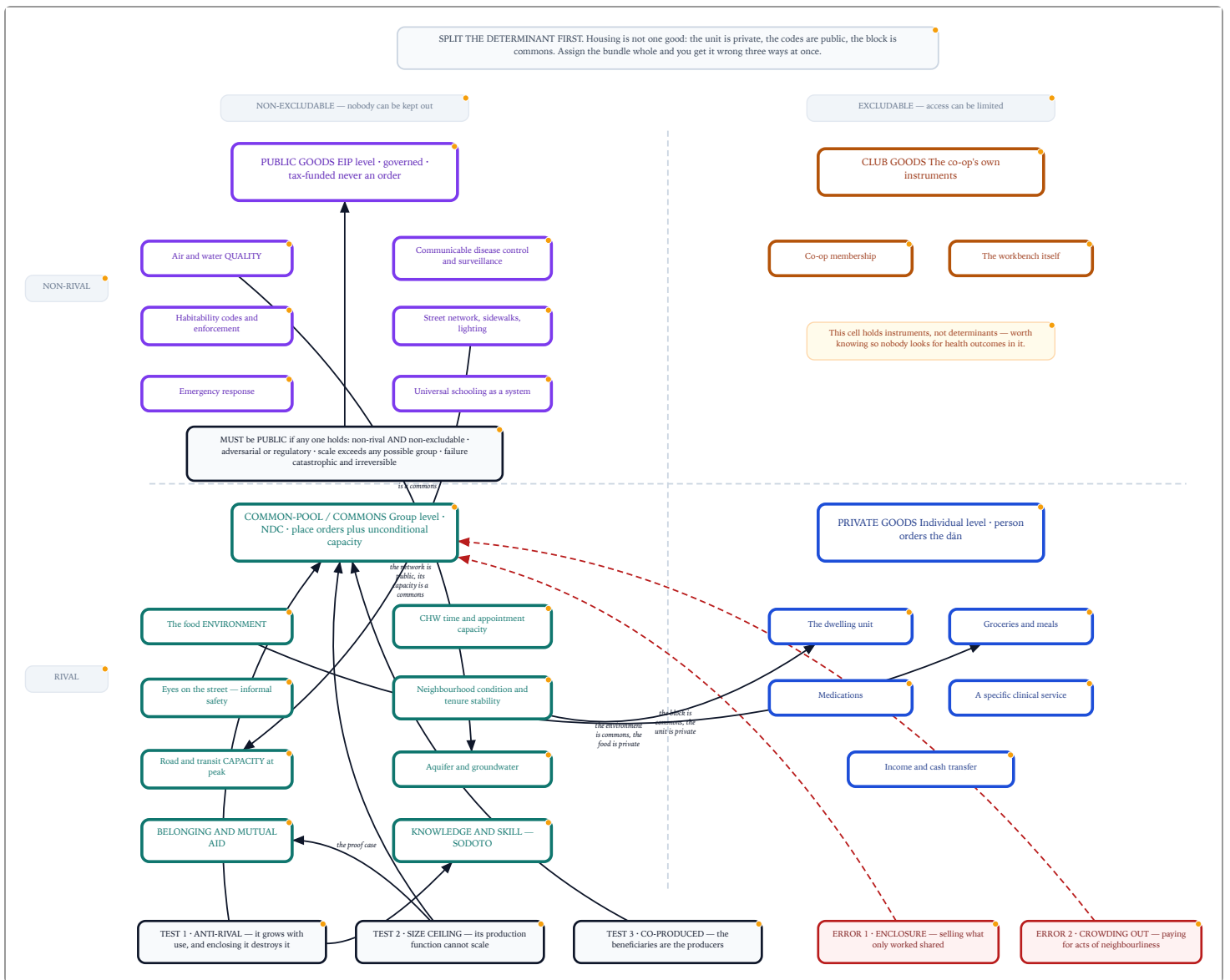


Figure 6 — Sorting the social determinants. Four cells on the two questions, with real determinants split across them. Purple is public, teal is commons, blue is private, amber is club. Black boxes are the tests that make something *necessarily* one or the other; red boxes are the two ways the sort gets broken.

PART 10

Reaching county scale without losing the neighborhood

IN ONE SENTENCE

Groups cannot grow without destroying what makes them groups, so they federate instead — and the whole question is whether the level above genuinely integrates what comes up to it, or merely counts votes.

Some things — a watershed, an airshed, communicable disease — exist far above the scale of any neighborhood. But growing a neighborhood group to county size destroys the belonging that let it act as one. Follett's answer, from 1918, is federation: you do not grow the group, you federate groups.

Everything then depends on a distinction that is easy to miss, because both versions look like meetings:

- **Aggregation** is the ballot. Delegates arrive with mandates, positions are counted, a majority forms, a minority loses. Difference was destroyed before anyone walked in.
- **Integration** runs the group process one level up and produces something no member brought with them.

Four rules make the difference, and one test detects it:

- **The relationship between levels is a loop, not an arrow.** Experience and disagreement go up; the integrated result comes back down and the group has to absorb it. If the return trip quietly stops, the delegate has become a politician.
- **Delegates report distributions, not positions.** A delegate brings the disagreement inside their neighborhood, not its settled view — because difference is the raw material the next level works with, and a mandate has already burned it.
- **The federation is itself a group,** so it obeys the same size law: roughly seven to twelve bodies, then another tier. Every extra tier costs a full return trip.
- **Rotation, a right to leave, and graduated sanctions.** Rotation prevents a professional class forming and spreads the skill, which is made in the doing. The last two come from Elinor Ostrom's work on how communities successfully govern shared resources.

THE TEST

Name one decision this body made that no member walked in with.
If you cannot, it is aggregating, and it should be described as a bargaining table rather than a federation.

How many neighborhoods?

The wrong question. A neighborhood organization exists wherever residents care to invest themselves in taking care of everyone in that place. It is not surveyed into existence and cannot be. So coverage will be uneven, sizes will differ, and groups will form, dissolve and reform. The seven-to-twelve rule tells you when a body must split — not how many neighborhoods Whatcom is supposed to have.

THE FAILURE MODE THAT COMES WITH EMERGENCE

Self-organization correlates with time, security, tenure and prior trust. So the fragmented, displaced and frightened places are the least likely to produce a group — and they are exactly the places with the worst health. Left alone, an emergent design concentrates its benefits where they were least needed, and it will look like people simply choosing not to participate.

The answer is not to go and organize them; an outsider convening a group produces a clientele. What transfers is presence, invitation and accompaniment, over years, before there is anything to fund. And the federation's share must be held *deliberately unequal* in favor of weaker groups, or advantage compounds.

ONE REASSURANCE

The person register is universal; the place register is emergent. A person order needs no group to verify it — only the person. So somebody living on a block that never organizes still has a plan, still has a care team, still signs their own confirmation. A coverage gap costs a neighborhood its commons work. It never costs an individual their care.

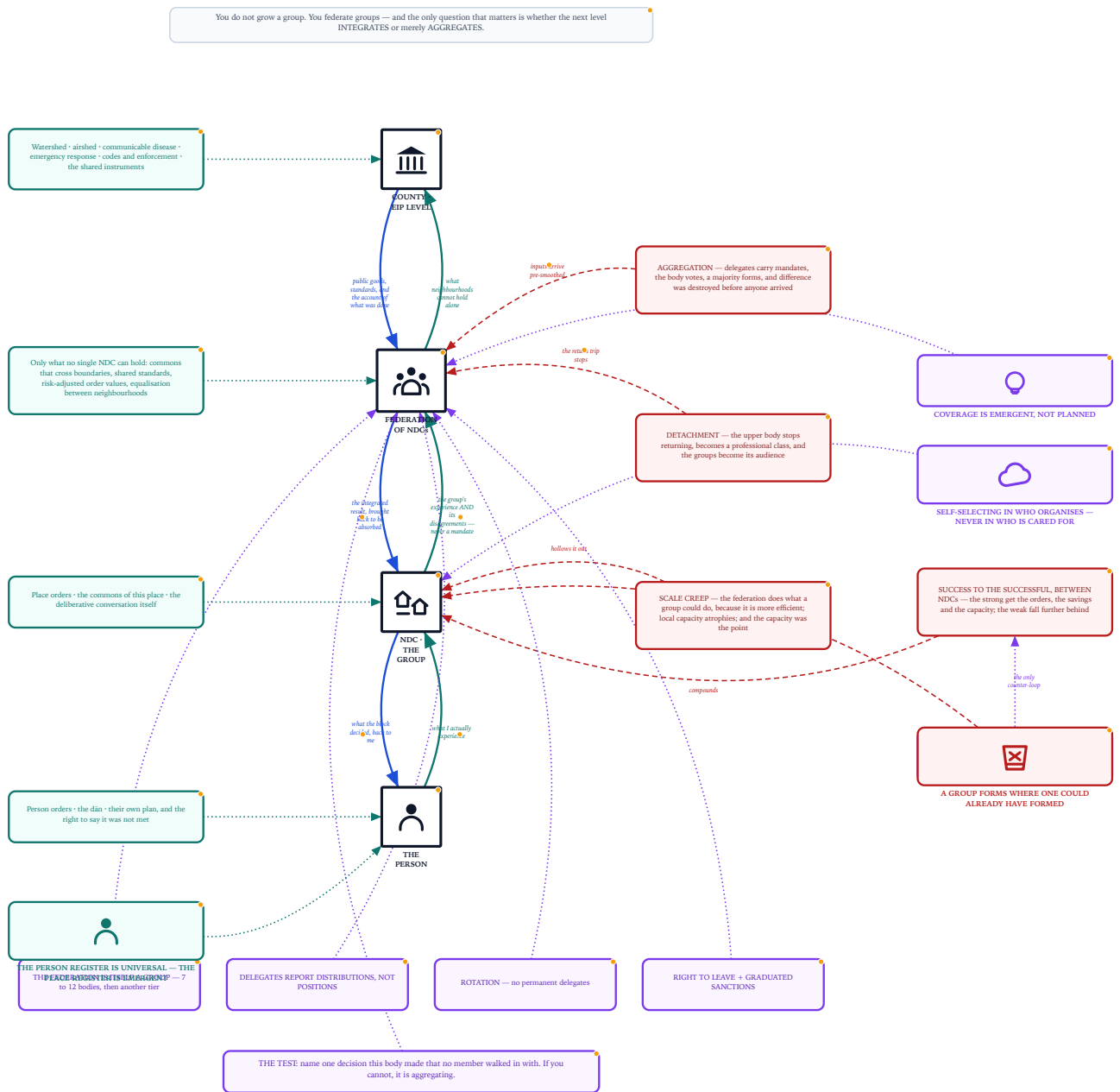


Figure 7 — Federation. Four levels down the middle, each pair joined by two arrows rather than one — experience going up, the integrated result coming back down. Teal on the left: what each level may hold, and nothing a level below could. Red on the right: how it fails. Purple: what makes it integrate rather than aggregate.

PART 11

What we don't know, and what would stop us

An honest list. Several of these are answerable quickly, and answering them is more valuable than any further design.

QUESTION	WHY IT MATTERS	HOW TO FIND OUT
What is the actual payer mix in Whatcom?	It determines which adapter matters most. This brief uses estimates, not local data.	County and state figures; the broker will likely know it from memory.
Is there an employer pool?	The fastest adapter depends on enough self-funded employers locally. No pool, no first foothold.	Ask the broker. Probably an afternoon's work.
Is it legal in this shape?	Stark and anti-kickback law constrain precisely what we want: outcome-contingent payments moving between parties who also refer to each other.	A health lawyer, engaged <i>before</i> instruments are drafted.
Can we survive a ratcheting benchmark?	The Medicare adapter punishes success with a harder target. Model several years, not one.	Actuarial modeling against real ACO experience.
Can we risk-adjust?	Without it the savings share compounds inequality while looking like merit. Medicare brings its own; the others may not.	Actuarial input; possibly the hardest technical piece.
Does Medicaid churn break the plan?	A plan needs continuity, and eligibility does not provide it.	Look at real churn rates in the local Medicaid population.
Will community-benefit dollars move?	It is the only realistic person-register funding for the uninsured.	A direct conversation with the hospital, using its own reporting obligations.
Will care teams hold an order?	The whole model assumes a small team will take responsibility for a named person's plan.	E1 tests exactly this, with nothing at risk.
Is verification honest?	If members learn what answer is wanted, the signal is worthless.	E2, plus independent sample calls.

QUESTION	WHY IT MATTERS	HOW TO FIND OUT
Do groups form where we need them?	The place register only exists where residents invest themselves.	Unknowable in advance. Watch, accompany, do not force.

Two open arguments the group should have on purpose

Does pay-at-risk belong here at all? Deming would object to the shared-upside stream on principle. The defense offered above is real but not obviously complete, and it should be settled consciously rather than absorbed by default. The stream could be set to zero and most of the design still works.

What is a plan a proxy for? A care plan stands in for what a person actually wants from their life. Align payment to the plan and everything in it gets attention — which means everything not in it becomes invisible in a new way. No stopping rule detects this, because the ledger will look excellent. The only defenses are that the person writes the plan, can rewrite it, and that "partly met" is a real answer they can give without cost.

REFERENCE

Glossary

Care team

Any small group a member actually meets — a clinic pod, a pharmacy, an imaging suite, a therapy practice, a community health worker team, a hospital service line.

Support team

A team a member never meets: billing, data, supply, training. Paid from capacity, and chargeable to the care teams that use it.

Adapter

The arrangement by which one payment group's money reaches the order register. Four of them: employer pool, ACO, delegated Medicaid arrangement, avoided cost.

ACO

Accountable care organization. Takes responsibility for the total cost and quality of care for an attributed group of Medicare beneficiaries, and shares in savings against a benchmark. No insurance license required.

Managed care organization (MCO)

A plan paid a fixed amount per member per month by the state to cover Medicaid enrollees.

ACH

Accountable Community of Health. Washington's regional bodies for cross-sector health improvement; Whatcom sits within one.

Community benefit

Spending a nonprofit hospital must make and report in exchange for its tax exemption. Money that is already committed, whose direction is negotiable.

Benchmark and attribution

The expected spending a shared-savings arrangement is measured against, and the rule deciding which people count as yours.

RenDanHeYi (人单合一)

Haier's management model. Loosely: merging the person with a concrete single order. Small self-managing units contract directly with named customers rather than receiving instructions.

Dān (单) / the order

One identified person's specific need. Here, a person's shared care plan — or, for public health, a named condition in a named place.

Microenterprise

The economic description of a care team: it has its own account and can accept or decline a piece of work.

Microcommunity

Several care teams bound into one contract around one person's outcome, so none can succeed while the outcome fails.

Control chart

A statistical tool distinguishing ordinary random variation from a genuine signal. Used here as a gate: a measure cannot become a payment trigger until charting

shows its movement means something.

Leading vs lagging measure

A leading measure is something a team can move this week (days to a first visit). A lagging one arrives long after and is confounded by everything (readmission rates).

Public good

Nobody can be kept out, and one person's use doesn't diminish another's. Free-riding means voluntary local provision fails, so these need public funding and governance.

Commons (common-pool resource)

Nobody can be kept out, but use *does* diminish it, so it can be exhausted. Governed well by neither market nor state alone. Elinor Ostrom's field.

Anti-rival good

Something that grows with use rather than being consumed — knowledge, skill, trust. Enclosing it is self-defeating, not merely unfair.

Wrong-pocket problem

The entity that pays for prevention is not the one that captures the savings, so prevention never gets funded.

Crowding out

Paying for something people were doing voluntarily replaces the norm with a transaction, and the transaction performs worse.

Integration vs aggregation

Follett's distinction. Aggregation counts pre-formed positions and makes a majority. Integration produces something no participant brought in.

Group vs crowd

A group interweaves difference; a crowd achieves false unity by suppressing it. Fear manufactures crowds; a crowd is not a large group but the destruction of one.

PDSA / the ramp

Plan-Do-Study-Act. Test a change at small scale, study what happened, widen one variable at a time. Never implement untested.

APPENDIX A

The self-funded employer adapter, in detail

WHY THIS ONE FIRST

It needs nobody's permission. No federal application, no state license, no waiver — only employers who already carry their own claims risk deciding to buy differently, and appointing the cooperative to do it for them.

A **self-funded** employer pays its employees' claims from its own funds, usually buying administration from a third party and catastrophic protection from a reinsurer. It already holds the risk. What it typically does not hold is control over what its money buys, which is delegated by default to a network and a fee schedule someone else designed.

What the cooperative would be

Contracting agent for a pool of such employers. It would:

- define the order — what it means for a member's plan to be met;
- contract directly with care teams, on the three streams in Part 6;
- hold the order register and the verification record;
- settle payments from the pooled fund, on a transparent administration fee;
- open work to whoever can meet it, and end arrangements that do not.

What it needs

COMPONENT	WHY
A pool of employers	The leverage is aggregate. One employer cannot move a care team's economics; a pool can.
A settlement rail	A third-party administrator or equivalent to move money and keep the ledger. Buildable, or buyable.

COMPONENT	WHY
Stop-loss cover	Not a technicality. It is what buys the experiment enough time to show anything, because one bad claims year would otherwise end it.
Plan-design control	Without it the cooperative can contract but cannot change what is covered, which is half the point.
A health lawyer, early	Stark and anti-kickback law constrain exactly the instrument shape we want. This shapes the contracts rather than reviewing them.

How it fails

Employers leave the pool after the first bad year. Everything else is recoverable; this is not, because the pool *is* the leverage. Two mitigations: stop-loss cover, and sequencing the experiments so that E0 and E1 produce visible information before anyone's costs change.

How it connects to the others

Deliberately, this adapter is not a silo. The order register it fills is the same register the Medicare and Medicaid adapters fill, and the care teams it contracts are the same teams. So when an employee turns 65 and moves to Medicare, or loses a job and moves to Medicaid, their plan is already in the register and the care team already holds it. Only the money's route changes. That continuity is not a bonus feature — it is the reason to build one workbench rather than four programs.

REFERENCE

The diagrams

All seven figures are live files. They open in the RCN Graph Tool ([tools/graph-tool-v22.html](https://tools.rcn.org/graph-tool-v22.html)) via Import → JSON, where every box carries a hover note explaining the reasoning behind it — roughly 170 notes across the set, considerably more detail than this document contains.

```
tools/premium-position.rcn.json  – Figure 1
tools/four-adapters.rcn.json    – Figure 2
tools/coop-fork.rcn.json        – Figure 3
tools/money-and-performance.rcn.json – Figure 4
tools/experiment-ramp.rcn.json  – Figure 5
tools/sdoh-sorting-frame.rcn.json – Figure 6
tools/federation.rcn.json       – Figure 7
```

Sources behind this brief: W. Edwards Deming, and Langley, Moen, Nolan, Nolan, Norman and Provost, *The Improvement Guide*; Norman, Provost, Moen and colleagues, *Quality as an Organizational Strategy*; Zhang Ruimin and Haier's RenDanHeYi model; Mary Parker Follett, *The New State* (1918); Elinor Ostrom on governing the commons; Geoffrey Rose on the prevention paradox; and the Nuka, Esther and Buurtzorg systems. Figures drawn in the RCN Graph Tool. This is a proposal for argument — the parts most likely to be wrong are flagged as such throughout.